

RADIOLOGY IMAGING REPORT

Royal General Hospital — Department of Radiology

PATIENT NAME: Maria Santos
DATE OF BIRTH: 08/11/1962 (Age: 63 years)
REFERRING PHYSICIAN: Dr. Kevin Okafor
REPORTED ON: 30 April 2026

PATIENT ID: PT-2026-06615
GENDER: Female

EXAMINATION DATE: 30 April 2026
ACCESSION NO.: RAD-2026-44821

EXAMINATION DETAILS

Modality: Plain Radiography (X-Ray)

Study: Chest — Posteroanterior (PA) and Left Lateral Views

Clinical Indication: Progressive dyspnoea, persistent cough × 6 weeks, bilateral ankle oedema. Known history of hypertension and ischaemic heart disease.

Comparison: Previous chest X-ray dated 14 January 2026

TECHNICAL QUALITY

Adequate inspiratory effort. Proper PA positioning confirmed. Good contrast and exposure. Both lung fields and costophrenic angles are visible. No significant rotation.

RADIOLOGICAL FINDINGS

Heart and Mediastinum:

Cardiothoracic ratio is 0.62, indicating cardiomegaly (increased from 0.57 on prior study). The cardiac silhouette is globular in contour, raising concern for pericardial effusion. Superior mediastinum is not widened. Aortic arch is mildly unfolded and calcified, consistent with atherosclerosis.

Lung Fields:

Bilateral perihilar haziness and vascular engorgement consistent with pulmonary venous hypertension. Kerley B lines visible at the right base, suggesting interstitial oedema. No focal consolidation, mass lesion, or cavitation identified. Ill-defined opacification in the bilateral lower zones, more pronounced on the right, in keeping with bilateral pleural effusions — right greater than left.

Pleura:

Bilateral pleural effusions — moderate on the right, small on the left. Both costophrenic angles are blunted. No evidence of pneumothorax.

Bones and Soft Tissues:

Thoracic cage is intact with no rib fractures or lytic lesions identified. Mild degenerative changes noted in the thoracic spine. Soft tissues are unremarkable. Old healed fracture noted at right 6th rib (chronic appearance).

Diaphragm and Abdomen:

Both hemidiaphragms are mildly elevated. Gaseous distension of the stomach. No free subdiaphragmatic gas.

IMPRESSION / CONCLUSION

PRIMARY FINDINGS:

- 1. Cardiomegaly (CT ratio 0.62) — increased from prior study. Pericardial effusion cannot be excluded; echocardiography recommended.
- 2. Bilateral pleural effusions (right > left) with features of congestive cardiac failure.
- 3. Pulmonary venous hypertension with interstitial oedema (Kerley B lines, perihilar haziness).
- 4. Unfolded calcified aortic arch consistent with systemic hypertension and atherosclerosis.
- 5. Old healed right 6th rib fracture — incidental finding, no acute rib pathology.

RECOMMENDATIONS:

Urgent cardiology review. Transthoracic echocardiogram to assess LV function and rule out pericardial effusion. Consider BNP / NT-proBNP. Clinical correlation required. If no improvement with diuresis, CT chest with contrast may be warranted to exclude underlying malignancy.

